

SCHIZOPHRENIA

- **Ensure a specialist confirms the diagnosis of schizophrenia.**

- Consider schizophrenia in the patient who (if no mental health or alcohol/drug disorder) has for at least 6 months had difficulty carrying out ordinary work, domestic or social activities *and* for at least 1 month has had ≥ 2 of the following symptoms of psychosis:
 - Delusions: unusual/bizarre beliefs not shared by society; beliefs that thoughts are being inserted or broadcast.
 - Hallucinations: usually hearing voices or seeing things that are not there.
 - Disorganised speech: incoherent or irrelevant speech
 - Behaviour that is disorganised or catatonic (inability to talk, move or respond) *or* negative symptoms: lack of emotion or facial expression, no motivation, not moving or talking much, social withdrawal.

Assess the patient with schizophrenia

Assess	When to assess	Note
Symptoms	Every visit	• Assess symptoms of psychosis above. If symptoms of psychosis and: - Aggressive/violent ↗ 84. - Varying levels of consciousness over hours/days and/or temperature ≥ 38°C, delirium likely ↗ 85. - Patient has interrupted treatment: restart intramuscular treatment ↗ 147 and explore reasons for poor adherence (like side effects, substance misuse) ↗ 173. - Good adherence to optimal doses of treatment, discuss/refer. • Manage other symptoms as on symptom pages.
Self-harm	Every visit	If patient has suicidal thoughts or plans, refer same day ↗ 83. If intent to harm others, alert intended victim/s if possible.
Stressors	Every visit	Help identify stressors that may worsen or cause symptoms to recur. If patient is being abused ↗ 88.
Alcohol/drug use	Every visit	In the past year, has patient: 1) drunk ≥ 4 drinks ¹ /session, 2) used illegal drugs or 3) misused prescription or over-the-counter medications? If yes to any ↗ 142.
Family planning	Every visit	Assess patient's contraceptive needs ↗ 154. If patient is pregnant, planning pregnancy or breastfeeding, refer to specialist.
Medication	Every visit	• Ask about treatment side effects ↗ 147. • Ask about adherence. If non-adherent, restart medication at same dose, explore reasons for stopping treatment and refer for community health worker support. • Discuss with specialist if patient is on medication that might cause acute psychosis, like prednisone, efavirenz, moxifloxacin and terizidone.
Weight (BMI)	Every visit	• BMI = weight (kg) ÷ height (m) ÷ height (m). • If gaining weight, refer to dietician if available and discuss with specialist about possible alternative schizophrenia treatment.
Glucose	At diagnosis, then yearly	If known diabetes ↗ 130. If not known with diabetes, check glucose ↗ 17.
Random total cholesterol	At diagnosis, then 2 yearly	• Assess and manage CVD risk ↗ 127. • If cholesterol increasing, discuss with specialist about possible alternative schizophrenia treatment.
HIV	At diagnosis or if status unknown	Test for HIV ↗ 110. If HIV positive, avoid efavirenz, discuss treatment with specialist.
Syphilis	At diagnosis	If positive, treat ↗ 53 and refer.

¹One drink is 1 tot of spirits, or 1 small glass (125mL) of wine or 1 can/bottle (330mL) of beer.